

ACUTE FLACCID PARALYSIS

1. Definition: AFP= Acute flaccid paralysis

This is any flaccid paralytic manifestation observed children under the age of 15, regardless of vaccination status.

What cases must be notified?

- Sudden flaccid or limp paralysis of one or more limbs in a healthy child.
- Paralysis sets in progressively over 3 days and becomes flaccid, whatever the cause (trauma, injections, etc....).
- Trauma can trigger a paralysis that was incubating.
- May begin with fever but fever may go undetected if febrile.
- Can reach 1 member, 2 members or 4 members.
- Paralysis may be symmetrical or asymmetrical.
- Impotence of the affected limb is observed.
- There is paresis or muscle weakness.
- Non-traumatic gait disorders such as lameness.
- Loss of movement or muscle tone.

If functional impotence is advanced, there is muscle weakness.

NOTIFICATION / Health information circuit

Notify the Health District of any cases observed:

- District Manager
- District PFA focal point
- The case must appear in the register of a health facility
- Notify the District and/or the PFA Focal Point before sending samples.
- The copy of the EPI-numbered form must be filed in the structure.
- The other copy is to be given to the Health District for filing.

Response: If a case of polio has been confirmed, vaccinate all children under the age of 15 within 4 weeks; local vaccination throughout the hillside. Organize door-to-door vaccination in three rounds, one month apart. The national program will take the lead.

2. Clinical signs

Mothers often come to the clinic because their children longer sit up or walk.

On examination, we find :

- flaccid paralysis of one or more limbs (usually one leg), with amyotrophy and abolition of reflexes
- normal sensitivity
- a few days before: illness with fever, diarrhea, vomiting Diagnostic

test:

Lay the child on his back with his legs bent and his feet flat on the table; the affected leg falls if you let go of your knees.

The PEC can be found on following page

3. Case management

- * Symptomatic treatment of fever and diarrhea
- * Rest and nursing
- * Ban IM injections
- * Referral as soon as possible (as soon as pain subsides and within 7 days of onset) for KINESITHERAPY and functional rehabilitation (2-3 week hospital stay), for :
 - * prevent vicious retractions
 - * reduce paralysis
 - * consider a fitting

4. Action to be taken

- * Vaccination
- * Food hygiene and drinking water; sanitation
- * Information Education Communication (IEC)
- * Sample to be sent to the laboratory
- * Monitor vaccination coverage.